



PROCEDURE

Visitors – Relatives, Carers, Personal Support Persons and Friends

Scope (Staff):	All NMHS Mental Health (MH) Inpatient Staff
Scope (Area):	All NMHS MH Inpatient Areas

1. Aim

Maintaining contact with family, friends and carers is important during an inpatient stay. It is crucial that safe and efficient processes are established to ensure patients who wish to have visitors are given the opportunity to do so in a safe environment. This procedure stipulates the requirements and processes for managing visitors to a NMHS Mental Health Inpatient Unit.

2. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☒	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Definitions

Visitor: Any person, being a relative, carer, personal support person (PSP), friend or a professional, religious or legal person who visits the patient in hospital in accordance with the patient's wishes.

Visiting room: refers to either a designated visitor's room/area or pre-approved interview room assessed by the clinical team as suitable for the purpose of visiting.

Carer: Defined by [Carers Recognition Act 2004 \(WA\)](#) as an individual who provides ongoing care, support and assistance to a person with disability, a chronic illness (which includes mental illness) or who is frail, without receiving a salary or wage for the care they provide.

Child: Anyone under 18 years of age.

Contraband: Items which are prohibited or unauthorised and may be used by a patient to harm themselves or others. Examples of these may be:-

- cigarette lighters, matches

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- vapes
 - drug paraphernalia
 - alcohol
 - batons, crossbows, flick knife/switchblade, knuckle dusters
 - spray weapons (including pepper spray)
 - swords, martial arts weapons
 - unidentified substances in any form include, but not limited to:
 - tablets
 - pastes
 - plant materials
 - liquids
 - powders
 - crystalline

4. Processes

- Relatives, carers, Personal Support Persons (PSP) and friends will be encouraged to visit.
- Patients, relatives, carers, PSP's and friends are to be given full information on visiting procedures, along with other relevant policies such as property, alcohol, smoking etc., on admission by the ward nursing staff.

Refer to:

- [DoH Smoke Free Policy](#)
- [NMHS Smoke Free Policy](#)
 - [NMHS MH Smoking Care Guideline](#)
- [NMHS Carers Policy](#)
- [NMHS MHS Search of Person, Property, Environment and Seizure of Property Procedure](#)
- [NMHS MHS Alcohol and Illicit Substances – Management of](#)
- Each ward will clearly display their normal visiting hours.
- Visiting hours outside of these hours may be arranged with the Clinical Nurse Specialist (or delegate) who will determine if alternate requested times are suitable.
- Visitors, who reside outside of the metropolitan area may be granted alternative visiting access as agreed to by the treating team and approved by the Head of Clinical Service or Coordinator of Nursing.
- Visitors under the age of 18 years must be accompanied by a responsible adult unless approved by the Head of Clinical Service (HOCS) or Coordinator of Nursing.
- All visitors will be asked where possible if they have any concerns regarding the visit both pre and post visit by staff providing access to the visit and facilitating visitor/s leaving the ward.

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- Staff will check all items given to the patient for contraband or other hazards and record any property or cash received by the patient into the property sheet as per normal property process.
 - Visits should be recorded in the patient's Integrated Progress Notes.
 - Should clinical staff assess that visiting is not in the best interests of the patient, visitors will be informed and encouraged to reschedule their visit. This should be clearly documented and communicated as to the rationale for the decline of the visit at that time.

4.1 Patient rights

Mental Health Act 2014 (WA)

Under the [Mental Health Act 2014 \(WA\)](#) patients have a right to private, lawful and uncensored communication, such as being able to see and speak to others in hospital and being visited or contacted by others including legal representation.

Under the *Mental Health Act 2014 (WA)* the following applies to visitors:

- s. 261 (Austl.). a patient has the right of freedom of lawful communication in reasonable privacy, including receiving visits.
- s. 262 (Austl.). a psychiatrist can make an order restricting a patient's access to visitors if the psychiatrist is satisfied that making the order is in the best interests of the patient.

Identified significant Aboriginal family/community, which may include traditional healers, are strongly supported to be involved in Aboriginal patient's mental health care.

Visitors will be advised that the use of camera or video facilities on their mobile phones is strictly prohibited to ensure patient privacy is maintained. Refer to the [NMHS Photography, Filming and Recording by Patients, Carers, Relatives or Visitors](#).

4.2 Conditions of visitation

- All visits from a patient's Personal Support Person(s) and friends will be conducted in a manner that ensures the privacy and safety of all patients, visitors, and staff.
- Children 18 years of age and under, must be accompanied and supervised by a responsible adult, other than the patient, at all times while within the inpatient unit.
- In special situations where older children (aged between 16 and 18 years) wish to visit independently, the request will be individually assessed to determine the safety and appropriateness of an unaccompanied visit.
- Visitors with children, may need to wait in the reception area until the ward area is assessed by staff for suitability of access by children. If staff have a concern regarding the risks, safety, health or welfare of a visiting child, this should be reported and discussed with the team immediately. Following discussion with the team, concerns should be documented and a plan of action formulated, which may include reporting concerns to an external agency such as the [Department of Communities Child Protection Division](#).

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- Reception staff, where available, will telephone the relevant ward to inform staff of the visitor's arrival and/or direct the visitor(s) to the ward.
 - If the reception is unattended and/or the main entry door is locked, visitors will contact the ward via the telephone located at the front entry door and ward staff will facilitate access for the visitor.
 - Visitors are to utilise the specific visitor's room/area in each of the inpatient wards unless otherwise advised by the clinical staff.

4.3 Visiting children

The welfare of a child/children is paramount. If staff have concerns regarding the safety, health or welfare of a visiting child, this should be reported and discussed with the team immediately. Concerns following discussion should be documented in the patient's file and a plan of action formulated, which may include contacting the Department of Communities Child Protection Division as per the [WA Department of Health Guidelines for Protecting Children 2020](#).

4.4 Visits to locked wards/secure wards

- Visitation access to locked/secure wards for under 18 year old's is restricted.
- Visitors under 18 years of age are only granted access:
 - at the treating team's discretion,
 - the visit has been prearranged, and
 - a thorough risk assessment has been completed prior to their visit.
- Visitors should be encouraged to telephone the ward prior to their visit to ensure they have access to the visitors' room where available. This information is recorded in the Ward Diary and an alternative time negotiated should there be a clash with a pre-existing booking.
- Visitors should be informed that if there is a change in circumstance it may result in visitation being rearranged, and this may even occur on their arrival.

4.5 Accessing visits to locked wards/secure wards

- Reception staff, where available, should inform the ward staff about the arrival of visitors for the patient in a locked/secure ward.
- If locked/secure ward staff are concerned about the current mental state of the patient prior to a visit, the consultant psychiatrist should be informed. A decision to decline or restrict a visit and the reason for this decision is to be managed pursuant to the *Mental Health Act 2014* (WA) s. 262 (Austl.).
- The visiting room will be used for all visits unless otherwise indicated with clear documentation of the rationale.
- Access to other areas for the patient and their visitors will be determined by the treating team and documented in the patient's health record.
- If required, the patient will be placed on 15-minute visual observations during the visitation. This decision will be made by the treating consultant or delegate.



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- Prior to using the visiting room, ward staff are to ensure the security of the room.
 - Staff should provide visitors with an explanation regarding items which may be inappropriate to leave with a patient. Visitors may take their belongings with them, unless staff have reason to believe that the visitor is bringing in contraband items (e.g. illicit substances etc.).
 - Staff are to escort the visitor into an identified room first before bringing in the patient from the ward. When the visit has ended, staff will ask the patient if they have received any items from the visitor which may need to be assessed for property listing and risk. Staff should then escort the patient back to the ward before returning to escort the visitor(s) from the room.
 - In the event that a staff member reasonably suspects that an inappropriate item has been brought into an inpatient unit, refer to:
 - [MH Search of Person, Property, Environment and Seizure of Property Procedure](#)
 - [MH Alcohol, Illicit Substances and Non-Prescribed Substances Management of Procedure](#)
 - Objects considered to be unsafe are to be handed in by the visitor prior to the visit and stored in the property cupboard. These can be handed back to the visitor at the completion of their visit.
 - Any relevant information obtained or observed during the visit should be recorded in the patient's health record. This may include PSOLIS risk alerts about visitor and/or patient.

5. Older Adult Mental Health Service Visitors Access

Within Older Adult Mental Health at Selby Lodge (Selby) and Osborne Lodge (Osborne) there are similar procedures to follow when patients have visitors.

5.1 Selby Lodge - OAMHS Lower West Shenton Park

- During office hours, when visitors arrive at Selby Lodge, they will attend at the main entrance and check in with the receptionist, and sign the visitors' book. The receptionist will let them through the first door down to the hospital wings.
- If visitors are unsure of where to go, the receptionist can call the appropriate wing and a staff member will attend and orientate them showing them where to go.
- After hours, the visitors must come to the night entrance and press the bell, this alerts both A and B wings, the shift co-ordinator will look at the monitor and if appropriate let the visitors in.
- Visitors are required to sign the visitor's book, which is located near the night entrance door.
- When patients have visitors, they may access the visitor's rooms on both sides A and B of the wings.
- If there are no visitor's rooms available, visitors can sit at the dining room table and, weather permitting, utilise the courtyard.



5.2 Osborne Lodge - OAMHS Osborne Park Hospital G Block

- During office hours, when visitors arrive at Osborne Lodge, they will attend at to the main entrance and check in with the receptionist and sign the visitors' book. The receptionist will let them through the first door down to the hospital wings.
- If visitors are unsure of where to go, the receptionist will call the appropriate wing and a staff member will meet the visitors in reception and direct them where to go.
- After hours, the visitors must come to the night entrance and press the bell, which is activated in wing one, the staff have access to the camera to see the visitors before going to the night entrance doors to let them in.
- Visitors are required to sign the visitors' book which is located near the night entrance door.
- At Osborne Lodge, there are no visiting rooms. Visitors enter the ward area and have their visit in the patient's bedroom or if weather permitting outside in the courtyard or the solariums located in wing one.

6. Restricting a patient's access to visitors

An order to restrict a patient's access to visitors must be made using [Form 12C](#) – Restriction on Freedom of Communication.

A Form 12C, "*Restriction on freedom of communication*" will be completed (refer s. 262 (5) & (6) of the MHA 2014) by the Consultant Psychiatrist via PSOLIS and a copy filed in the medical record, and provided to the patient and to their carer, close family member and/or other personal support person/nominated persons by the Consultant Psychiatrist or delegate.

An order is current for 24 hours. After this time, the form will lapse. The order must be reviewed every 24 hours and the decision made by the Consultant Psychiatrist for the order to:

- Remain in force (for a further 24 hours),
- Be amended, or;
- Be revoked.

The decision will be documented on a [Form 12C.i](#) (SHMHMR987) and filed in the medical record and a copy provided to the patient and to their carer, close family member and/or other personal support person/nominated persons, by the Consultant Psychiatrist or delegate.

The Consultant Psychiatrist will document in the patient's Integrated Progress Notes (PMR7) explaining why the patient's freedom of lawful communication has been limited or restricted.

The Shift Coordinator will indicate in the ward report book or equivalent, that a patient's right to freedom of lawful communication has been limited or restricted.

Clinical staff will be responsible for removing personal mobile devices and restricting access to the ward public/other available means of communication for patients on restrictions.

6.1 Restriction for Visitors with Inappropriate, Offensive or Aggressive Behaviour

Visitors may present with inappropriate, offensive or aggressive behaviours. The service has the right to restrict or deny access to the unit to individuals, and this decision will be made by the Head of Clinical Service, Coordinator of Nursing or Service Manager in the best interests of patients and staff.

Examples of these may be:

- Aggressive behaviour towards staff in attendance, including threats to staff or property.
- Intoxicated individuals or groups who are displaying anti-social behaviours.
- Offensive behaviour, which includes a disregard for conditions of visit such as refusing to leave or swearing obscenities at staff or the patient.
- Displaying an inappropriate level of intimacy towards the patient (sitting on laps and excessive physical contact) compromising the reputation and wellbeing of the patient.
- Dealing and/or supplying illicit drugs/alcohol/smoking paraphernalia to the patient.

The offending visitor/s should be informed that their behaviour is deemed as unacceptable and if that behaviour continues then they will be asked to leave. If the behaviour continues the visitor will be escorted off the campus via security staff and/or depending on the behaviour, police may be contacted to attend. Some behaviour will warrant immediate ceasing of the visit by staff and if necessary any future visits will need to be discussed with the patient and the treating team.

This decision must be documented in the integrated notes of the patient the person is visiting and include:

- A full summary of the discussion and the information provided to the visitor during the discussion including any agreed outcomes/consequences.
- Names of senior staff members involved in discussion/decision.
- An OSH form should be logged for any such behaviour displayed by a visitor.

Refer to:

- [NMHS Reporting Workplace Violence & Aggression Incidents to WA Policy Guide](#)
- [NMHS OSH – Guide to Reporting Incidents of Workplace Violence & Aggression](#)

7. Compliance, monitoring and evaluation

- Clinical Incident Management System
- OSH Incident Review
- Consumer Feedback
- Chief Psychiatrists Standards for Clinical Care.

Related internal policies, procedures and guidelines

Department of Health

[MP 0158/21 Smoke Free Policy](#)

NMHS

[NMHS Smoke Free Policy](#)

[NMHS Carer's Policy](#)

[NMHS Reporting Workplace Violence & Aggression Incidents to WA Policy Guide](#)

[NMHS OSH – Guide to Reporting Incidents of Workplace Violence & Aggression](#)

[NMHS Security – Use of Force Policy](#)

Mental Health

[NMHS MH Search of Person, Property, Environment and Seizure of Property Procedure](#)

[NMHS MH Smoke Free Procedure](#)

[NMHS MH Smoking Care Guideline](#)

[NMHS MH Alcohol and Illicit Substances – Management of](#)

Useful resources

[The Western Australian Carers Charter](#)

[MHPHDS Consumer Feedback form](#)



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